



Modified Essay Questions

SCORING KEY

AUGUST 2018

INSTRUCTIONS:

- Please use pencil ONLY, preferably 2B
- Do not fold or bend
- Erase mistakes fully
- Make no stray marks
- Completely fill in the oval

Please MARK LIKE THIS ONLY:

① ② ③ ④

Modified Essay 5

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist employed at a community child and adolescent clinic.

A 14-year-old girl, Emily, is referred to your clinic with 15kg weight loss during the last 6 months. She describes increased food restriction over the last month, eating only leafy vegetables, and jogging for at least an hour a day. She has been amenorrhoeic for 3 months. She presents as an obviously thin girl (BMI is 15 kg/m²) wearing oversized clothing, who believes she is still overweight and would like to be thinner.

Question 5.1

Outline (list and justify) the key aims of the initial assessment.

Please note: a list with no justification will not receive any marks. (8 marks)

A.	IMMEDIATE RISK. Consider – medical risk, self-harming associated with psychological vulnerability and eating disorder behaviours. e.g. purging, laxatives or diuretics abuse, diet tablets.	① ②
B.	DEVELOPMENTAL AND DIAGNOSTIC ISSUES. Exclude other intercurrent conditions (e.g. anxiety, mood, cognitive effects) and understanding presentation on context of broader developmental and social context (e.g. school and social activity).	① ②
C.	LEGAL. Consider relevant legal framework	①
D.	PATIENT PERSPECTIVE. Clarify Emily's understanding of reason of attendance and assess capacity – influence on Emily's engagement in and collaboration with a difficult treatment process.	①
E.	FAMILY AND SOCIAL PERSPECTIVE. Consider family and social context, support, capabilities, attitudes etc. (e.g. parents response to Emily's weight loss; attitude to psychiatry; intercurrent family stressors).	① ②
F.	Treatment setting that balances patient/family preference and risk minimisation.	①
G.	Spare (only to be used after approval from Co-Chairs, Writtens Subcommittee)	①
H.	Did not attempt	○
I.	Did handwriting affect marking?	○

Note to Examiner: Please mark all bubbles even if the total adds up to more than 8.

Note to NDS: Please set the maximum mark to 8.

Marker ID

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The information that is presented in *italics* in this question is a repetition of the earlier sections of the case vignette.

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Question 5.2

Outline (list and justify) the specific physical findings and investigations that would indicate that hospital admission may be required.

Please note: a list with no justification will not receive any marks. (6 marks)

A.	Cardiac instability / deconditioning • Postural tachycardia (>20bpm increase) • Postural hypotension (> 10 mm drop) • HR < 40 bpm or > 120 • Systolic BP < 90	Cardiac muscle atrophy and autonomic instability and sudden cardiac death.	☐ 0 ☐ 1 ☐ 2
B.	Temperature – hypothermia < 35.5. or cold/blue extremities.	is associated with impaired thermoregulation and mortality risk	☐ 0 ☐ 1
C.	Blood glucose – hypoglycemia < 3.0.		☐ 0 ☐ 1
D.	Abnormal ECG: any arrhythmia, non-specific ST or T wave changes, prolonged QTC.	torsade de pointis	☐ 0 ☐ 1 ☐ 2
E.	Significant electrolyte abnormalities • Sodium < 130 • Hypokalaemia, • Hypophosphataemia	arrhythmia and refeeding	☐ 0 ☐ 1
F.	Other abnormal investigations • Neutropenia < 1.5 • Reduced albumin • Elevated liver enzymes	infection and organ failure	☐ 0 ☐ 1
G.	BMI < 14		☐ 0 ☐ 1



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Emily is admitted to a paediatric medical ward.

Question 5.3

Outline (list and justify) the strategies to reduce the risk of refeeding syndrome.

Please note: a list with no justification will not receive any marks. **(6 marks)**

A.	Treatment parameters. Multidisciplinary team with experienced dietician, regular review, index of suspicion – allow early identification and tailored response.	0 1
B.	Moderate pace of refeeding - “start low and go slow” (e.g. 6000kJ/1500kcal daily in an adult) due to risk of carbohydrate-induced insulin release triggering rapid intracellular electrolyte uptake – consider NG if risk of refeeding syndrome considered high – recognize risk of underfeeding.	0 1
C.	Low carbohydrate diet/preparations to avoid triggering postprandial rebound hypoglycaemia in people with low glycogen stores.	0 1
D.	Limit oral fluid intake, appropriate to age and weight because excess fluid intake may worsen electrolyte disturbance.	0 1
E.	Other electrolyte monitoring and supplementation. Potassium, Magnesium.	0 1
F.	Ongoing daily monitoring in first 1-2 weeks or until goal energy intake is reached: repeat LFTs, electrolytes, ECG, blood sugar.	0 1

G.	Spare (only to be used after approval from Co-Chairs, Wittens Subcommittee)	☒
H.	Did not attempt	☐

I.	Did handwriting affect marking?	☐
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Note to Examiner: Please mark all bubbles even if the total adds up to more than 6.

Note to NDS: Please set the maximum mark to 6.

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Emily is admitted to a paediatric medical ward.

Several weeks later, Emily has gained weight and is medically stable. You meet with Emily and her family for discharge planning.

During previous family meetings, Emily has complained that her parents control her life. Her father, a headmaster, is "never at home". Her mother is a successful interior designer and Emily says "Nothing I do is ever good enough for her". Emily says that her mother has always been preoccupied with her own appearance and weight.

Question 5.4

Outline (list and justify) the treatment approaches that you would consider discussing with Emily and her parents.

Please note: a list with no justification will not receive any marks. (6 marks)



1. ☐ 2. ☐ 3. ☐ 4. ☐ 5. ☐ 6. ☐ 7. ☐ 8. ☐ 9. ☐ 10. ☐ 11. ☐ 12. ☐ 13. ☐ 14. ☐ 15. ☐ 16. ☐ 17. ☐ 18. ☐ 19. ☐ 20. ☐ 21. ☐ 22. ☐ 23. ☐ 24. ☐ 25. ☐ 26. ☐ 27. ☐ 28. ☐ 29. ☐ 30. ☐ 31. ☐ 32. ☐ 33. ☐ 34. ☐ 35. ☐ 36. ☐ 37. ☐ 38. ☐ 39. ☐ 40. ☐ 41. ☐ 42. ☐ 43. ☐ 44. ☐ 45. ☐ 46. ☐ 47. ☐ 48. ☐ 49. ☐ 50. ☐ 51. ☐ 52. ☐ 53. ☐ 54. ☐ 55. ☐ 56. ☐ 57. ☐ 58. ☐ 59. ☐ 60. ☐ 61. ☐ 62. ☐ 63. ☐ 64. ☐ 65. ☐ 66. ☐ 67. ☐ 68. ☐ 69. ☐ 70. ☐ 71. ☐ 72. ☐ 73. ☐ 74. ☐ 75. ☐ 76. ☐ 77. ☐ 78. ☐ 79. ☐ 80. ☐ 81. ☐ 82. ☐ 83. ☐ 84. ☐ 85. ☐ 86. ☐ 87. ☐ 88. ☐ 89. ☐ 90. ☐ 91. ☐ 92. ☐ 93. ☐ 94. ☐ 95. ☐ 96. ☐ 97. ☐ 98. ☐ 99. ☐ 100. ☐

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